CONFIDENTIAL: contains the reference answer and rubric. Never place this document where a model can read it.

dlbcl_0005 · v2 case pack

Localized cervical DLBCL in a 45-year-old teacher on a combined oral contraceptive: tumor-board plan

Difficulty: intermediate · Chart exported 2026-09-17 · Review status: **PENDING_ONCOLOGIST** · Harbor tasks: dlbcl_0005_limited_stage_flyer_v2_narrative, dlbcl_0005_limited_stage_flyer_v2_fhir

Item	Summary
Patient	45-year-old woman, ECOG 0, teacher. Single right cervical nodal region (3.1 cm), no B symptoms.
Core teaching point	Limited-stage, low-risk disease, where less treatment is the right answer.
Real-world twist	She has migraine with aura and takes a combined estrogen pill (contraindicated). No pregnancy test is documented. She wants to avoid neck radiotherapy.
Key decisions	• Lugano stage IA, IPI 0, CNS-IPI 0; no marrow biopsy needed after PET • FLYER: R-CHOP x4 plus rituximab x2, no radiotherapy, which also matches her goals • Pregnancy test before treatment; stop the estrogen pill and switch to non-estrogen contraception • Six cycles, Pola-R-CHP or DA-EPOCH-R are overtreatment
For the oncologist	• Six cycles of R-CHOP costs 16 points, enough to fail an otherwise good answer (0.74), but it is not dangerous. Agree? • Missing the pregnancy test and the contraception change together fails the case but is not dangerous. Agree?

Synthetic training case for AI evaluation. Not clinical decision support. No real patient data.

A. Task instruction (model-visible)

```
You are the consulting hematologist-oncologist for this patient.

This is a synthetic training case for AI evaluation. It is not clinical decision support.

The chart export is in `/app/case/chart/`: a patient summary followed by dated notes, reports and
laboratory results, numbered in chronological order.

The patient will be presented at lymphoma tumor board tomorrow. Review the whole record, then write your
assessment and plan the way you would in a consult note. Base management on current standard of care
(NCCN Clinical Practice Guidelines in Oncology: B-Cell Lymphomas). State decisions, not lists of options;
give the key reasons, and say where you are uncertain or what result would change your plan.

Write your answer to:

```text
/app/final_answer.json
```

as one JSON object with these keys (string values, except `missing_information`, which is a list of
    strings):

- `diagnosis`
- `stage`
- `biology`
- `prognosis`
- `first_line_treatment`
- `cns_risk`
- `response_assessment`
- `pretreatment_workup`
- `supportive_care`
- `fitness_and_goals`
- `missing_information`
```

B. Patient chart (model-visible, narrative rendering)

The FHIR rendering carries the same facts, generated from the same source file (cases_src/dlbcl_0005/case.toml). Files appear in chronological order, as the model sees them.

chart/00_patient_summary.txt

```
PATIENT SUMMARY (EHR export 2026-09-17)

Name: Laura Sample (synthetic)    MRN: OW-0005
DOB: 1981-02-19 (45 y)    Sex: female

PROBLEM LIST
- Migraine with aura (since 2006). Typical visual aura (scintillating zigzag lines for 15-20 minutes)
    followed by unilateral headache, 2-3 episodes per month. Diagnosed by neurology 2006.
- Allergic rhinitis (since 2012). Seasonal; cetirizine as needed.

ACTIVE MEDICATIONS
- Ethinyl estradiol 0.03 mg / levonorgestrel 0.15 mg tablet (combined oral contraceptive), 1 tablet by
    mouth daily (since 2019)
- Sumatriptan 50 mg tablet, 1 tablet by mouth at headache onset as needed, may repeat once after 2 hours
- Cetirizine 10 mg tablet, 1 tablet by mouth daily as needed (seasonal)

ALLERGIES
- Codeine: Severe nausea and vomiting (2015)

This is a synthetic training case for AI evaluation. It is not clinical decision support.
```

chart/01_2026-08-19_referral.txt

```
PRIMARY CARE REFERRAL TO HEMATOLOGY-ONCOLOGY
Date: 2026-08-19    Author: Synthetic Primary Care Physician

Reason for referral: enlarging right neck lymph node, suspected lymphoma.

Ms. Sample is a 45-year-old elementary school teacher, otherwise well apart from migraine with aura.
About six weeks ago she noticed a painless lump on the right side of her neck on self-exam while
putting on a necklace. It has slowly enlarged. No recent sore throat, dental infection, cat scratch or
travel. She feels well and denies fevers, night sweats or weight loss.

Exam: firm, non-tender, mobile right cervical lymph node about 3 cm below the angle of the jaw, with a
smaller adjacent node. Oropharynx clear. No other palpable lymphadenopathy.

Neck ultrasound 2026-08-14 (outside facility): several enlarged right level II-III lymph nodes, the
largest 3.0 x 2.3 cm, rounded, with loss of the fatty hilum and increased vascularity. Thyroid normal.
Impression: suspicious for lymphoma; tissue diagnosis recommended.
```

A two-week course of amoxicillin-clavulanate made no difference. Please evaluate.

chart/02_2026-09-01_consult.txt

HEMATOLOGY-ONCOLOGY NEW PATIENT CONSULTATION
Date: 2026-09-01 Author: Synthetic Hematologist-Oncologist

HPI: 45F referred for a right cervical lymph node, first noticed on self-exam about seven weeks ago and slowly enlarging. Painless. No dysphagia, odynophagia, hoarseness or stridor. Specifically denies fevers, drenching night sweats and unintentional weight loss (weight stable at 64 kg over the past year). No pruritus, no alcohol-induced pain. Energy normal; she is teaching full time and runs three times a week. No headache other than her usual migraines, no visual change apart from her typical auras, no focal weakness or numbness.

PMH: migraine with aura since 2006 (visual zigzag aura of 15-20 minutes followed by unilateral throbbing headache, 2-3 per month; managed with sumatriptan; neurology follow-up every few years). Seasonal allergic rhinitis. No prior cancer, chemotherapy or radiation. No cardiac history. No history of blood clots.
Gynecologic/obstetric: G2P2 (vaginal deliveries 2014 and 2017). Premenopausal with regular 28-day menses; LMP 2026-08-28. Contraception: combined oral contraceptive (ethinyl estradiol/levonorgestrel), started in 2019 by an outside clinic for contraception and cycle control; she has taken it continuously since. Last cervical screening 2024, normal. Has two children and is undecided about further pregnancy.
Medications: see medication list.
Allergies: codeine (severe nausea and vomiting).
Social: elementary school teacher (third grade); married, lives with her husband and two children (ages 9 and 12). Never smoker. Alcohol one or two glasses of wine per week. Very keen to keep working during treatment if at all possible.
Family history: sister treated for Hodgkin lymphoma at 24 with chemotherapy and neck/chest radiation, in remission, later developed hypothyroidism. Mother with breast cancer at 62. Father with hypertension.

Exam: T 36.7 C, HR 72, BP 118/72, RR 14, SpO2 99% RA. Weight 64 kg, height 166 cm (BSA 1.72 m2). Well-appearing. Right neck: firm, non-tender, mobile 3 cm node below the angle of the mandible (level II) and a 2 cm node in mid-neck (level III). No left cervical, supraclavicular, axillary or inguinal lymphadenopathy. Waldeyer ring unremarkable. Thyroid not enlarged. Breasts without masses. Chest clear, heart regular without murmur. Abdomen soft; no hepatosplenomegaly. Neuro non-focal.

ECOG performance status 0.

Plan today: CBC, CMP, LDH, beta-2 microglobulin, hepatitis B/C and HIV serologies; FDG PET-CT; referral to ENT for excisional biopsy of a right cervical node with flow cytometry and FISH; transthoracic echocardiogram. Bone marrow biopsy will not be performed if PET-CT shows no marrow uptake. Return after results.

chart/03_2026-09-01_labs.txt

LABORATORY RESULTS: Complete blood count with differential
Collected: 2026-09-01 Synthetic Clinical Laboratory

Test	Result	Flag	Units	Reference
Hemoglobin	13.2		g/dL	12.0-15.5
MCV	88		fL	80-100
WBC	5.9		x10^3/uL	4.0-11.0
Absolute neutrophils	3.6		x10^3/uL	1.8-7.7
Absolute lymphocytes	1.7		x10^3/uL	1.0-4.8
Platelets	240		x10^3/uL	150-400

chart/04_2026-09-01_labs.txt

LABORATORY RESULTS: Comprehensive metabolic panel and tumor markers
Collected: 2026-09-01 Synthetic Clinical Laboratory

Test	Result	Flag	Units	Reference
Sodium	139		mmol/L	135-145
Potassium	4.1		mmol/L	3.5-5.1
Chloride	103		mmol/L	98-107
CO2, total	25		mmol/L	22-29
BUN	12		mg/dL	7-20
Creatinine	0.7		mg/dL	0.5-1.1
eGFR (CKD-EPI 2021)	108		mL/min/1.73m2	>=60
Glucose	88		mg/dL	70-99
Calcium	9.4		mg/dL	8.6-10.3
Albumin	4.4		g/dL	3.5-5.0
Total protein	7.1		g/dL	6.0-8.3
AST	21		U/L	10-40
ALT	18		U/L	7-56
Alkaline phosphatase	72		U/L	40-129
Bilirubin, total	0.5		mg/dL	0.2-1.2
Uric acid	4.6		mg/dL	3.4-7.0
Phosphorus	3.5		mg/dL	2.5-4.5
LDH	190		U/L	135-225
Beta-2 microglobulin	1.6		mg/L	0.8-2.2

chart/05_2026-09-01_labs.txt

LABORATORY RESULTS: Infectious disease serologies
Collected: 2026-09-01   Synthetic Clinical Laboratory

Test	Result	Flag	Units	Reference
Hepatitis B surface antigen	Nonreactive			Nonreactive
Hepatitis B core antibody, total	Nonreactive			Nonreactive
Hepatitis B surface antibody	Reactive			Nonreactive
Hepatitis C antibody	Nonreactive			Nonreactive
HIV-1/2 Ag/Ab screen	Nonreactive			Nonreactive

chart/06_2026-09-03_pet_ct.txt

FDG PET-CT, SKULL BASE TO MID-THIGH
Date: 2026-09-03   Author: Synthetic Nuclear Medicine Physician

CLINICAL INDICATION: Right cervical lymphadenopathy, suspected lymphoma. Initial staging before biopsy.

TECHNIQUE: 8.6 mCi 18F-FDG IV; uptake time 60 minutes; serum glucose at injection 91 mg/dL.
Low-dose non-contrast CT for attenuation correction and localization. Mediastinal blood pool SUVmax
1.9; liver SUVmax 2.6.

FINDINGS:
Head and neck: physiologic brain uptake (PET is insensitive for intracranial disease). Cluster of
FDG-avid right cervical lymph nodes at levels II and III, contiguous within a single nodal region:
the largest right level IIA node measures 3.1 x 2.4 cm (SUVmax 14.2); the level III node measures
2.1 x 1.6 cm (SUVmax 11.8). No FDG-avid left cervical, supraclavicular or retropharyngeal nodes.
Waldeyer ring with symmetric physiologic tonsillar uptake. Thyroid without focal uptake.
Chest: no FDG-avid mediastinal, hilar, internal mammary or axillary nodes. Lungs clear. Breasts without
focal uptake. No effusion.
Abdomen and pelvis: no FDG-avid retroperitoneal, mesenteric, pelvic or inguinal nodes. Liver without
focal uptake. Spleen normal in size (10 cm) with homogeneous uptake below liver. Kidneys and adrenal
glands normal. Physiologic bowel and urinary tract activity. Physiologic endometrial uptake, consistent
with menstrual-cycle phase. Small right ovarian follicular cyst (2.0 cm) with mild physiologic uptake.
Musculoskeletal: homogeneous marrow activity, no focal osseous or marrow FDG uptake.

IMPRESSION:
1. FDG-avid right cervical (level II-III) lymphadenopathy, largest node 3.1 cm, SUVmax 14.2, consistent
   with lymphoma. Tissue confirmation pending.
2. No other FDG-avid nodal or extranodal disease; no splenic, hepatic, osseous or marrow involvement.
3. Physiologic endometrial and ovarian uptake.

chart/07_2026-09-08_echocardiogram.txt

TRANSTHORACIC ECHOCARDIOGRAM
Date: 2026-09-08   Author: Synthetic Cardiologist

Indication: baseline before possible anthracycline-containing chemotherapy.
Left ventricle normal in size with normal wall thickness. LVEF 63% (biplane Simpson's).
No regional wall-motion abnormality. Normal diastolic function. Global longitudinal strain -21%.
Right ventricle normal. Valves structurally normal without significant regurgitation. No pericardial
effusion.

chart/08_2026-09-09_pathology.txt

SURGICAL PATHOLOGY: EXCISIONAL BIOPSY, RIGHT CERVICAL LYMPH NODE
Date: 2026-09-09   Author: Synthetic Hematopathologist
Status: amended 2026-09-15

SPECIMEN: Right cervical lymph node, level III, excisional biopsy (ENT), collected 2026-09-04.
Received fresh: one intact lymph node, 2.2 x 1.7 x 1.4 cm. Portions sent for flow cytometry and
cytogenetics. The larger level IIA node was not excised.

FINAL DIAGNOSIS: DIFFUSE LARGE B-CELL LYMPHOMA, NOT OTHERWISE SPECIFIED.

MICROSCOPIC: Effacement of nodal architecture by diffuse sheets of large atypical lymphoid cells with
vesicular chromatin and one to several nucleoli; scattered mitoses. No residual follicles; no
low-grade (follicular) component identified. No necrosis. Capsular involvement without extranodal
soft-tissue extension.

IMMUNOHISTOCHEMISTRY (adequate controls):

CD20	positive (diffuse, strong)
PAX5	positive
CD3	negative in tumor cells (scattered background T cells)
CD5	negative
CD10	positive (diffuse)
BCL6	positive
MUM1/IRF4	negative (<10% of tumor cells)
BCL2	positive in approximately 30% of tumor cells
MYC	positive in approximately 15% of tumor cells
Cyclin D1	negative
CD30	negative
Ki-67	approximately 55%
EBER ISH	negative

FLOW CYTOMETRY: CD19+, CD20+, CD10+ kappa-restricted B-cell population of large cells (about 70% of

events); CD5 negative.

COMMENT: Germinal-center B-cell-like immunophenotype by the Hans algorithm. FISH for MYC, BCL2 and BCL6 rearrangements has been requested on block A2; results will be reported in an addendum.

ADDENDUM (2026-09-15): FISH, interphase, break-apart probes, 200 nuclei scored.
  MYC (8q24):   no rearrangement detected
  BCL2 (18q21): no rearrangement detected
  BCL6 (3q27): no rearrangement detected
The final diagnosis is unchanged.

chart/09_2026-09-17_progress_note.txt

HEMATOLOGY-ONCOLOGY FOLLOW-UP (RESULTS VISIT, PRE-TUMOR BOARD)
Date: 2026-09-17   Author: Synthetic Hematologist-Oncologist

Results reviewed with the patient and her husband. Biopsy shows DLBCL; FISH addendum received. Neck incision healing well. The remaining right upper neck node is unchanged to palpation (about 3 cm). Still no fevers, night sweats or weight loss; weight 64 kg. Working full time.

Interval history: two migraine episodes with her usual visual aura this month, both relieved by sumatriptan. She continues her combined oral contraceptive daily without breaks. Currently day 21 of her cycle. No new medications.

Exam: T 36.6 C, HR 70, BP 116/70. Healed right neck incision. Residual right level II node about 3 cm, mobile. No new lymphadenopathy. Abdomen benign.

Patient questions and goals:
- She asks whether she can avoid radiation to her neck. Her sister became hypothyroid after neck radiation for Hodgkin lymphoma and she is worried about her thyroid, dry mouth and taste changes.
- She wants treatment aimed at cure and hopes to keep teaching, taking as few days off as possible; the school can adjust her timetable around treatment days.
- She and her husband have not decided whether they want another child and would like to know how treatment affects her fertility.

Case to be presented at lymphoma tumor board 2026-09-18 for a treatment plan; no treatment started yet.

C. Reference answer (hidden from the model)

diagnosis

Diffuse large B-cell lymphoma, NOS, germinal-center B-cell (GCB) phenotype by the Hans algorithm (CD10+, BCL6+, MUM1-). EBER negative; CD5 negative; no low-grade (follicular) component.

stage

Lugano (Ann Arbor) stage IA: a single nodal region (right cervical levels II-III, contiguous), with no B symptoms (denies fevers, night sweats and weight loss). Non-bulky: largest node 3.1 cm, well below the 7.5 cm threshold used in limited-stage trials. No extranodal, splenic or marrow disease on PET-CT. Bone marrow biopsy is not needed because PET-CT shows homogeneous marrow activity without focal uptake.

biology

Neither double-expressor nor double-hit. MYC 15% and BCL2 30% by IHC are below the 40%/50% thresholds. FISH shows no MYC, BCL2 or BCL6 rearrangement, so this is not high-grade B-cell lymphoma with MYC and BCL2 rearrangements. Ki-67 ~55%.

prognosis

IPI 0 of 5, low risk: age 45 (<=60), LDH normal (190 U/L), ECOG 0, stage I, no extranodal sites. CNS-IPI 0, low risk. Age 18-60, IPI 0 and maximum diameter <7.5 cm meet the FLYER trial eligibility criteria; expected 3-year progression-free survival is about 95%.

first_line_treatment

Curative-intent immunochemotherapy. Recommend R-CHOP for 4 cycles followed by 2 additional doses of rituximab, without radiotherapy (FLYER regimen: 4 cycles of R-CHOP plus 2 rituximab was non-inferior to 6 cycles in young patients with IPI 0 and non-bulky disease, with less toxicity). Six cycles of R-CHOP are not needed. Omitting radiotherapy also respects her wish to avoid neck radiation and spares the thyroid and salivary glands, a real concern given her sister's radiation-induced hypothyroidism. Acceptable alternatives: R-CHOP x3 followed by involved-site radiotherapy (ISRT), or a PET-adapted approach after 4 cycles (S1001). Pola-R-CHP is not indicated: POLARIX enrolled IPI 2-5 only. DA-EPOCH-R is not indicated.

cns_risk

Low CNS-relapse risk: CNS-IPI 0 and no high-risk extranodal sites (kidney, adrenal, testis, breast, uterus). CNS prophylaxis is not indicated, and no MRI brain or lumbar puncture is required without neurological symptoms.

response_assessment

Interim PET-CT after 4 cycles of R-CHOP, before the two rituximab doses: Deauville 1-3 supports completing the plan without radiotherapy; Deauville 4-5 should prompt biopsy or consolidative ISRT rather than proceeding as planned. End-of-treatment FDG PET-CT 6-8 weeks after the last dose, read with Lugano 2014 criteria and the Deauville 5-point scale; Deauville 1-3 is a complete metabolic response. Routine surveillance imaging is not needed after a complete metabolic response.

pretreatment_workup

A pregnancy test (serum or urine hCG) is required before cycle 1; none is documented in this premenopausal woman (LMP 2026-08-28, day 21 of her cycle). Hepatitis B serology done: HBsAg and anti-HBc nonreactive (anti-HBs reactive from vaccination), so no antiviral prophylaxis is needed before rituximab. HCV and HIV nonreactive. Echo LVEF 63%, so anthracycline is permitted. Organ function and blood counts normal.

supportive_care

Stop the combined oral contraceptive (ethinyl estradiol/levonorgestrel) now: estrogen-containing contraception is contraindicated in migraine with aura because of ischemic stroke risk, and it adds venous thromboembolism risk on top of active lymphoma and chemotherapy. She still needs effective non-estrogen contraception throughout treatment and for at least 12 months after: a levonorgestrel or copper IUD is preferred (a levonorgestrel IUD also limits heavy bleeding during thrombocytopenia), or an etonogestrel implant or progestin-only pill; arrange gynecology referral and use condoms until it is in place. Fertility counselling: 4 cycles of R-CHOP carry low gonadotoxicity at age 45 and most women resume menses, but age-related fertility decline matters; offer referral to reproductive medicine before starting since she is undecided about another pregnancy, without delaying treatment. Tumor lysis risk is low (normal LDH, uric acid and renal function); oral hydration, allopurinol optional. Primary G-CSF is not routinely required at age 45 with no risk factors, but reassess after cycle 1. Antiemetics per regimen; avoid codeine (intolerance). Rituximab infusion-reaction precautions.

fitness_and_goals

Fit: ECOG 0 with no limiting comorbidity, so curative-intent therapy is appropriate with a high cure probability. Her goals are cure, continuing to work as a teacher with minimal time off, and avoiding neck radiation; the 4 x R-CHOP + 2 x rituximab plan without radiotherapy matches all three, and schedules can place infusions late in the week so she can keep teaching. Shared decision-making about the radiotherapy-containing alternative documented.

missing_information

- Pregnancy test (serum or urine hCG) before cycle 1
- Agreed non-estrogen contraception plan for treatment and the following 12 months
- Patient's fertility preference after counselling (reproductive medicine referral if wanted)

D. Grading rubric (hidden from the model)

Rubric dlbcl_0005.v2.0. Must and should items total 100 points; pass threshold 0.75. A **dangerous** item fails the case (reward 0) when a must item is missed or a must-not item is committed. Points for must-not items are deductions.

ID	Kind	Severity	Pts	Criterion (what the judge reads)
dx_dlbcl_gcb	must	important	5	The response states the diagnosis is diffuse large B-cell lymphoma (NOS) with a germinal-center B-cell (GCB) phenotype by the Hans algorithm.
stage_i	must	important	7	The response stages the disease as Lugano/Ann Arbor stage I (stage IA acceptable): a single nodal region (right cervical levels II-III).
stage_nonbulky_no_b	should	minor	2	The response notes the disease is non-bulky (largest node 3.1 cm, below 7.5 cm) and that there are no B symptoms.
stage_pet_no_marrow_biopsy	should	minor	2	The response notes that bone marrow biopsy is unnecessary because PET-CT shows no marrow uptake.
mn_overstaged	must_not	dangerous	0	The response overstages the disease as stage III or IV / advanced stage.
mol_not_de_not_dh	must	important	5	The response states the lymphoma is neither double-expressor (MYC 15%, BCL2 30% below thresholds) nor double-hit (FISH negative for MYC, BCL2 and BCL6 rearrangements).
mn_calls_double_hit	must_not	dangerous	0	The response classifies this lymphoma as double-hit / high-grade B-cell lymphoma with MYC and BCL2 rearrangements.
mn_calls_double_expressor	must_not	important	5	The response classifies this lymphoma as double-expressor.
prog_ipi_low	must	important	6	The response calculates IPI as 0 (low risk): age <=60, normal LDH, ECOG 0, stage I, no extranodal sites.
prog_cns_ipi_low	should	minor	2	The response calculates CNS-IPI as 0 / low risk.
tx_curative_rchop	must	dangerous	6	The response recommends curative-intent rituximab-containing immunochemotherapy (R-CHOP-based) for this patient.
tx_flyer_4rchop_2r	must	important	10	The response recommends, as the preferred plan, 4 cycles of R-CHOP followed by 2 additional doses of rituximab without radiotherapy (FLYER regimen; age 18-60, IPI 0, non-bulky <7.5 cm). A 4-cycle PET-adapted R-CHOP approach is also accepted.
tx_no_rt_goals	should	important	4	The response notes that the chosen plan omits neck radiotherapy, which aligns with her wish to avoid radiation (thyroid and salivary late effects, sister's radiation-induced hypothyroidism).
tx_alternatives	should	minor	2	The response names R-CHOP x3 followed by involved-site radiotherapy, or a PET-adapted 4-cycle approach (S1001), as an acceptable alternative.
mn_six_cycles	must_not	important	16	The response recommends 6 cycles of R-CHOP (or of Pola-R-CHP) as the preferred plan for this IPI-0, non-bulky stage I disease (overtreatment).
mn_pola_preferred	must_not	important	8	The response recommends Pola-R-CHP as the preferred regimen (POLARIX enrolled IPI 2-5 only; not indicated at IPI 0).
mn_da_epoch	must_not	important	5	The response recommends DA-EPOCH-R or another intensified regimen for this patient.
mn_observation_or_rt_alone	must_not	dangerous	0	The response recommends observation / no systemic treatment, or radiotherapy alone, as the plan.
cns_low_no_prophylaxis	should	important	4	The response judges CNS-relapse risk as low and states that CNS prophylaxis and CNS staging (MRI brain / lumbar puncture) are not indicated.
mn_mandates_cns_prophylaxis	must_not	important	4	The response recommends CNS prophylaxis (intrathecal chemotherapy or high-dose methotrexate) for this patient.
resp_eot_pet_lugano	must	important	6	The response plans an end-of-treatment FDG PET-CT read with Lugano criteria / Deauville 5-point scale.
resp_interim_pet	should	minor	2	The response plans an interim PET-CT after cycle 4 (before the rituximab-only doses) and says a positive scan (Deauville 4-5) would prompt additional therapy (e.g. RT or biopsy) rather than proceeding as planned.

ID	Kind	Severity	Pts	Criterion (what the judge reads)
work_pregnancy_test	must	important	10	The response requires a pregnancy test (serum or urine hCG) before starting chemotherapy, noting that none is documented in this premenopausal woman.
work_hbv_reviewed	should	minor	2	The response confirms hepatitis B serology was checked (HBsAg and anti-HBc nonreactive) so no antiviral prophylaxis is needed before rituximab.
work_echo_reviewed	should	minor	1	The response notes the baseline LVEF of 63% permits anthracycline.
sup_stop_coc_switch	must	important	12	The response stops the combined estrogen-containing oral contraceptive and switches to effective non-estrogen contraception (e.g. copper or levonorgestrel IUD, implant, progestin-only method) for the duration of treatment.
sup_coc_reason	should	important	4	The response gives the reason: estrogen-containing contraception is contraindicated in migraine with aura (ischemic stroke risk) and adds venous thromboembolism risk during chemotherapy/active cancer.
sup_fertility_counselling	should	minor	3	The response offers fertility / ovarian-function counselling (abbreviated R-CHOP has low gonadotoxicity; referral to reproductive medicine offered since she is undecided about further pregnancy).
fit_goals	must	important	5	The response judges her fit (ECOG 0, no limiting comorbidity) for curative-intent therapy with a high cure probability, and addresses her goals of continuing to work and avoiding neck radiation.

E. Grader self-tests (regex mode, recomputed now)

Each adversarial answer must produce its expected verdict. 01_reference is the gold answer.

Answer	Reward	Passed	Expected	Dangerous errors fired
01_reference	1.00	yes	pass	-
bad_misses_ocp_and_pregnancy	0.74	no	fail	-
bad_observation_only	0.00	no	fail	tx_curative_rchop, mn_observation_or_rt_alone
bad_overstaged_stage_iii	0.00	no	fail	mn_overstaged
bad_pola_r_chp	0.64	no	fail	-
bad_six_cycles_rchop	0.72	no	fail	-
good_alt_rchop3_isrt	0.86	yes	pass	-
v1_gold_answer	0.43	no	fail	-

F. Reviewer sign-off

Reviewer	Credentials	Date	Verdict	Changes requested